

Amgen Strategic Analysis Report

Report Title: Calendar Year (CY) 2026 End-Stage Renal Disease (ESRD) Prospective Payment System Proposed Rule - CMS-1830-P

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Okay, Mr./Ms. CEO and Board of Directors, here is a comprehensive strategic report analyzing the proposed Calendar Year (CY) 2026 End-Stage Renal Disease (ESRD) Prospective Payment System (PPS) rule (CMS-1830-P) and related developments from the perspective of Amgen's Chief Strategy Officer.

EXECUTIVE SUMMARY

This report analyzes the proposed Medicare policy changes for End-Stage Renal Disease (ESRD) facilities and Acute Kidney Injury (AKI) dialysis services in CY 2026, as detailed in the CMS-1830-P proposed rule [Source 1: [federalregister.gov](https://www.federalregister.gov), Source 2: [federalregister.gov](https://www.federalregister.gov)]. Key proposed developments include updates to the ESRD PPS base rate, wage index, and outlier policy; a new payment adjustment for non-contiguous areas; modifications to the ESRD Quality Incentive Program (QIP); and, most notably for Amgen, a change to the Transitional Drug Add-On Payment Adjustment (TDAPA) eligibility timeframe and the termination of the mandatory ESRD Treatment Choices (ETC) Model.

These developments carry significant strategic implications for Amgen, particularly given our substantial presence in the nephrology therapeutic area. While the proposed payment rate increase for facilities is modest, changes to TDAPA eligibility could impact future pipeline product launches, and the termination of the ETC Model signals a shift in CMS's approach to value-based care in ESRD. Additionally, external insights highlight increasing operational challenges in accessing official regulatory information from sources like [FederalRegister.gov](https://www.federalregister.gov), posing a potential risk to timely compliance and strategic response.

Primary Opportunities:

- * Opportunity to engage CMS and provide feedback during the public comment period on the proposed rule, particularly concerning the TDAPA provisions and RFI topics (health IT, future QIP measures).
- * Potential for slightly improved financial stability for dialysis providers (Amgen customers) due to the modest payment update, though this is offset by QIP penalties.
- * Engagement opportunity regarding RFI topics on future quality metrics and technology adoption in dialysis.

Primary Threats:

- * Modification of TDAPA eligibility to a 3-year post-FDA approval window (effective 2028) could constrain the timeframe for securing favorable reimbursement and market access for future pipeline products targeting the ESRD population, potentially impacting launch sequence and commercial strategy.

* Termination of the ETC Model removes a specific lever designed to encourage home dialysis and transplant, potentially slowing the shift to alternative modalities which could indirectly impact Amgen's product utilization mix over the long term, depending on how our portfolio is positioned across modalities.

* Operational burden and risk associated with restricted automated access to critical regulatory information, potentially delaying awareness of final rule details and other important policy changes.

* QIP penalties continue to pressure the financial performance of dialysis providers, potentially impacting their capacity for technology adoption or investment in new therapies.

Recommended Strategic Priorities and Timeline:

1. Immediate (0-3 months): Form a cross-functional task force. Conduct detailed analysis of TDAPA impact on specific pipeline assets. Develop and submit comprehensive comments to CMS on the proposed rule, focusing on TDAPA and areas of RFI interest. Enhance regulatory intelligence monitoring capabilities to address access challenges. Engage key dialysis provider customers to understand their perspective on the proposed changes.

2. Medium-term (3-12 months): Integrate final rule provisions into market access and commercial strategies for relevant pipeline and marketed products. Develop internal training materials for Commercial and Medical Affairs teams. Implement enhanced regulatory monitoring processes. Monitor competitive responses and market dynamics post-ETC termination.

3. Long-term (12-24+ months): Refine portfolio strategy based on anticipated changes in the ESRD reimbursement landscape and modality trends. Continuously monitor CMS/CMMI strategy for ESRD and kidney care. Explore opportunities related to future QIP measures or health IT integration as they develop.

Critical Regulatory Change Classification Summary: The proposed rule contains several key ****Policy Changes**** and a notable ****Regulation Change**** (TDAPA timeline). Most are classified as ****Major/Monitoring**** while proposed, requiring close attention and strategic adaptation upon finalization. The TDAPA change is a ****Major**** impact on future product strategy. The regulatory access issue is a ****Major**** operational challenge.

REGULATORY CHANGE CLASSIFICATION & ANALYSIS

Based on the CMS-1830-P proposed rule [Source 1: [federalregister.gov](https://www.federalregister.gov), Source 2: [federalregister.gov](https://www.federalregister.gov)]:

Change Type Identification & Regulatory Impact Severity:

1. ESRD PPS Base Rate Update (1.9% market basket increase):

* **Change Type:** Policy Change (Adjustment to reimbursement methodology/rate).

* **Regulatory Impact Severity:** Major/Monitoring (Impacts the financial health of dialysis providers, our key customers, but is a proposed annual update within the existing PPS structure. Requires monitoring through final rule).

2. Geographic Wage Index & Outlier Payment Policy Updates:

* **Change Type:** Policy Change (Refinement of existing payment adjustments).

* **Regulatory Impact Severity:** Major/Monitoring (Impacts specific facility payments; requires monitoring for finalization).

3. Non-Contiguous Areas Payment Adjustment (NAPA):

* **Change Type:** Policy Change (New specific payment adjustment).

* **Regulatory Impact Severity:** Minor/Monitoring (Budget-neutral, limited geographic scope; requires monitoring).

4. Transitional Drug Add-On Payment Adjustment (TDAPA) Eligibility Timeframe Modification (3 years post-FDA approval, effective Jan 1, 2028):

* **Change Type:** Regulation Change (Formal modification of eligibility criteria/rule).

* **Regulatory Impact Severity:** Critical/Monitoring (While effective in 2028, this is a fundamental change to a key market access pathway for *future* drugs in the ESRD bundle. Requires critical analysis *now* and monitoring for final rule). *This is the most significant regulatory detail in the rule for Amgen's pipeline strategy.*

5. Continuation/Updates to Existing TDAPA/Post-TDAPA Payments:

* **Change Type:** Policy Change (Ongoing implementation of existing policy for specific drugs).

* **Regulatory Impact Severity:** Minor (Routine application of existing policy).

6. AKI Dialysis Payment Rate Alignment with ESRD PPS:

* **Change Type:** Policy Change (Alignment of two related payment systems).

* **Regulatory Impact Severity:** Minor/Monitoring (Streamlines payment; requires monitoring).

7. ESRD Quality Incentive Program (QIP) Updates (Measure Removal/Updates):

* **Change Type:** Policy Change (Modification of performance program criteria).

* **Regulatory Impact Severity:** Major/Monitoring (Influences provider behavior and can result in payment penalties; requires monitoring).

8. Termination of the Mandatory ESRD Treatment Choices (ETC) Model (effective Dec 31, 2025):

* **Change Type:** Policy Change (Termination of a specific value-based care model).

* **Regulatory Impact Severity:** Major (Significant shift away from this specific VBC approach in ESRD; impacts strategic positioning related to care modalities).

9. RFIs on Health IT, Interoperability, Future QIP Measures:

* **Change Type:** Policy Change (Signaling future policy direction and seeking input).

* **Regulatory Impact Severity:** Monitoring (Not binding, but crucial for identifying potential future policy changes and areas for engagement).

10. Regulatory Access Challenges (FederalRegister.gov access):

* **Change Type:** Policy Change (Administrative access policy).

* **Regulatory Impact Severity:** Major (Operational impact on compliance, legal, and regulatory teams; risk of delayed intelligence).

CROSS-FUNCTIONAL IMPACT ASSESSMENT

Affected Business Functions:

* **Regulatory Affairs:**

* ***Submission strategy modifications required:** * Yes, future TDAPA applications for kidney-related drugs must adhere to the new 3-year post-FDA approval window (effective 2028). This may necessitate earlier preparation of market access data packages.

* **New compliance obligations and timelines:** Yes, need to track the final rule carefully, understand the precise effective date and transition rules for TDAPA. Operational compliance burden for accessing regulatory information via new methods.

* **Regulatory pathway implications:** Minor implication for regulatory **approval** pathway (FDA), but major implication for **reimbursement/market access** pathway via TDAPA. Need coordination between Regulatory and Market Access.

* **Clinical Development:**

* **Protocol modifications needed:** Unlikely directly required by **this** rule. The TDAPA change might **indirectly** influence future trial design **if** specific endpoints or data requirements become crucial for demonstrating value within the constrained 3-year TDAPA window, though standard endpoints for efficacy and safety are primary.

* **New endpoint requirements or study designs:** See above. RFIs on future QIP measures (well-being, nutrition, physical activity) **could** inform future exploratory endpoints or research areas.

* **Patient safety and monitoring changes:** No direct impact from this rule.

* **Manufacturing & Quality:**

* **CMC (Chemistry, Manufacturing, Controls) updates:** No direct impact from this rule.

* **Quality system modifications:** No direct impact from this rule on product quality systems. Potential indirect impact on internal quality systems for regulatory intelligence monitoring processes.

* **Supply chain and vendor qualification impacts:** No direct impact from this rule.

* **Commercial Operations:**

* **Marketing and promotional claim adjustments:** Potential minor adjustments to messaging if referencing VBC models (due to ETC termination) or discussing quality metrics (due to QIP changes). Messaging around future products will need to incorporate the nuances of the modified TDAPA pathway.

* **Sales force training requirements:** Yes, significant training needed on the details of the proposed (and final) rule, including payment updates, QIP changes, TDAPA modification, and ETC termination, to effectively communicate with dialysis providers.

* **Market access and reimbursement strategy changes:** **Major Impact.** Need to revise market access strategies for pipeline products targeting ESRD to align with the 3-year TDAPA window (effective 2028). This impacts pricing strategy, evidence generation planning, and launch timing considerations. Need to model the financial impact of payment changes and QIP penalties on dialysis provider customers. Termination of ETC requires re-evaluation of strategies promoting home dialysis/transplant linked to that model.

* **Medical Affairs:**

* **Medical information updates:** Need to be prepared to discuss policy changes (TDAPA, ETC) with healthcare providers, though focus remains primarily on clinical data.

* **Healthcare provider communication needs:** Need clear, compliant messaging for Medical Science Liaisons (MSLs) and other field teams to address provider questions on how these policy changes affect their practice and patient care.

* **Post-market surveillance modifications:** No direct impact from this rule.

* **Legal & Compliance:**

* **Contract modifications with partners/vendors:** Review any existing contracts potentially linked to the ETC Model (unlikely significant, but prudent). Review vendor contracts for regulatory intelligence/monitoring services in light of access changes.

* **Compliance program updates:** Update training materials for Commercial and Medical teams. Review internal processes for regulatory monitoring and compliance with the new access environment.

* **Risk assessment and mitigation strategies:*** Update risk assessments related to market access for future products (TDAPA), operational compliance (regulatory access), and potential shifts in care modalities (ETC termination).

SCOPE DETERMINATION ANALYSIS

Product Scope Assessment:

*** Marketed Products:**

* **Which Amgen products are directly affected:*** Primarily our nephrology portfolio, including ESA products (Epogen, Aranesp) and products for secondary hyperparathyroidism (Sensipar/Parsabiv). These products are typically purchased or administered by dialysis facilities and are often included within the ESRD bundle payment or have separate payment pathways influenced by facility economics.

* **Required labeling or indication changes:*** None directly required by this payment rule.

* **Post-market commitment modifications:*** None directly required by this rule.

*** Pipeline Products:**

* **Development program adjustments needed:*** Potential minor influence on timing or data generation strategy to support future TDAPA submissions within the new 3-year window (effective 2028). This necessitates close coordination between R&D, Regulatory, and Market Access.

* **Regulatory submission timeline impacts:*** No direct impact on FDA submission timelines, but significant impact on **timing of TDAPA applications** relative to FDA approval for drugs applicable to the ESRD bundle.

* **Clinical trial protocol modifications:*** Unlikely directly, but RFIs on outcomes like well-being could inform future endpoint considerations.

*** Biosimilar Portfolio:**

* **Reference product comparability implications:*** None directly related to this ESRD rule.

* **Interchangeability pathway effects:*** None directly related to this ESRD rule.

* **Market entry strategy adjustments:*** Minor indirect impact if dialysis provider economics significantly shift, but the core strategy for biosimilars in this space is less tied to TDAPA or VBC models like ETC compared to novel bundled products.

Process Scope Assessment:

* **Manufacturing Processes:** No direct impact.

*** Quality Systems:**

* **QMS (Quality Management System) updates:*** Potential minor updates related to regulatory intelligence monitoring processes to ensure timely receipt and dissemination of critical information.

* **Documentation and training requirements:*** Documentation of new regulatory monitoring procedures. Training for relevant personnel on accessing information and tracking changes.

* **Audit and inspection preparedness:*** Unlikely direct impact, but regulators expect companies to be aware of and compliant with applicable rules; delayed awareness due to access issues could pose a theoretical risk.

*** Business Processes:**

* **Standard Operating Procedure updates:*** SOPs for regulatory intelligence, market access planning for pipeline assets, and potentially commercial/medical engagement with dialysis

providers may require updates.

* **Cross-functional workflow modifications:** Enhanced collaboration required between R&D, Regulatory, Market Access, Commercial, and Legal to manage the TDAPA timeline change and ETC termination implications.

* **Technology system adaptations:** Potential need to explore and implement new technical solutions or integrate with official APIs for regulatory monitoring, or implement more robust manual processes.

REMEDIATION LIFECYCLE MANAGEMENT

The remediation process will focus on strategic adaptation to the policy changes, enhancing regulatory intelligence, and adjusting market access strategies for future products.

Phase 1: Assessment & Planning (0-3 months - Current Focus)

* **Gap Analysis:** Assess current pipeline market access plans against the new 3-year TDAPA window (effective 2028). Evaluate current regulatory intelligence processes against the new access challenges. Analyze the strategic implications of ETC termination on Amgen's positioning in ESRD care modalities.

* **Resource Requirement Identification:** Identify personnel needed for the cross-functional task force. Assess potential need for investment in new regulatory intelligence tools or services. Estimate costs associated with enhanced manual monitoring.

* **Timeline and Milestone Development:** Establish a clear timeline for submitting comments on the proposed rule. Set milestones for final rule analysis, impact modeling, strategy adjustments, and internal training.

* **Cross-Functional Team Formation:** Establish a dedicated task force comprising leads from Regulatory Affairs, Market Access, Commercial, Medical Affairs, Legal & Compliance, and potentially R&D; (for pipeline implications). Regulatory Affairs or Market Access could lead, depending on primary focus. Steering committee oversight from Strategy and relevant Therapeutic Area/Commercial leadership.

Phase 2: Implementation & Testing (3-12 months - Post-Final Rule)

* **Remediation Activities:** Finalize updated market access strategies for affected pipeline assets. Develop internal training modules. Implement revised regulatory monitoring processes and systems. Develop communication plans for external stakeholders (dialysis providers, patient groups).

* **Remediation Testing:** Test new regulatory intelligence systems/processes for effectiveness and timeliness. Validate financial models assessing impact on providers and Amgen. Validate updated training materials. *Note: This phase is less about traditional manufacturing/quality testing for this type of policy change.*

Phase 3: Engagement & Adaptation (6-18 months)

* **Submission Strategy:** Focus shifts from comment submission (Phase 1) to integrating regulatory requirements (like TDAPA timing) into *future* product launch strategies. This includes planning for data generation to support TDAPA applications within the new window for applicable future drugs.

* **Submission Preparation:** Preparing necessary documentation and justification for *future* TDAPA submissions as relevant pipeline products approach approval, ensuring they meet the

criteria within the stipulated timeframe (effective 2028).

* **International Regulatory Alignment:** While a US-specific rule, insights gained from US implementation may inform strategies in other markets with similar bundled payment models, though direct alignment is unlikely.

Phase 4: Implementation & Monitoring (12-24 months)

* **Go-Live Activities:** Implement finalized strategies, including training rollout to relevant teams, launch preparations incorporating the new TDAPA reality, and ongoing engagement with stakeholders based on the final rule.

* **Post-Implementation Monitoring:** Continuously monitor the impact of the rule on dialysis provider financial health and purchasing patterns. Track CMS activities regarding ESRD policy and potential new VBC models. Assess the impact of ETC termination on modality shifts. Monitor competitive strategies in response to these changes. Evaluate the effectiveness of new regulatory monitoring processes.

AMGEN BUSINESS IMPACT ASSESSMENT

Direct Impact on Amgen:

* **Core therapeutic area implications:** Significant impact on our Nephrology portfolio (ESA's, shPT therapies). While existing products are not directly subject to new TDAPA rules, sales volume is tied to dialysis provider viability and patient volume/modality. The modest PPS increase is slightly positive for provider finances, but QIP penalties offset this. ETC termination removes a policy push towards home dialysis/transplant, which *could* impact the demand mix for products used differently across modalities (e.g., oral vs. IV administration, specific drug regimens).

* **Biosimilar business and pipeline effects:** Minimal direct impact on our biosimilar portfolio from this rule.

* **Manufacturing and supply chain considerations:** No direct impact.

* **Financial impact estimation:**

* ***Costs:** Costs associated with enhanced regulatory intelligence systems/processes. Costs of internal strategy development, training, and stakeholder engagement.

* ***Revenue:** Potential long-term impact on revenue from **future** pipeline products targeting ESRD due to a constrained TDAPA window (effective 2028). This is difficult to quantify precisely now as it depends on specific future products. Short-term impact on marketed products is likely limited, perhaps marginal due to provider financial changes. ETC termination impact is complex; depends on modality shift speed, which was limited even **with** the model.

* ***Timeline:** Accelerated timeline needed for market access preparations for future ESRD pipeline assets.

Subsidiary and Partnership Impact:

* Review any partnerships or collaborations focused on ESRD care delivery, value-based care models, or technology integration (e.g., health IT for dialysis) to assess alignment or conflict with the ETC termination and RFI topics. Licensing agreements for kidney-related assets should be reviewed for any clauses potentially affected by changes in reimbursement pathways like TDAPA.

COMPETITIVE LANDSCAPE ANALYSIS

Competitor Impact Assessment:

- * **How developments affect key competitors:** The proposed rule impacts all competitors with a significant presence or pipeline targeting the ESRD space (e.g., Vifor Pharma/CSL Behring, Akebia Therapeutics, Cara Therapeutics, Sanofi, etc.).
- * The **TDAPA timeline change** (effective 2028) is a systemic change affecting **all** future innovative products eligible for this pathway. It creates a level playing field in terms of the access window but pressures all companies to integrate this constraint into their development and launch planning.
- * The **payment updates and QIP changes** affect the financial health of the shared customer base (dialysis facilities), impacting all suppliers.
- * The **ETC termination** affects all companies with strategies potentially linked to promoting specific modalities within that model structure.
- * **Competitive advantage/disadvantage analysis:** No obvious immediate advantage or disadvantage is created by the **content** of the rule itself. Success will depend on how effectively each company adapts its pipeline strategy, market access planning (especially for TDAPA), and engagement with dialysis providers and CMS. The operational challenge of regulatory access affects all large, regulated companies, and those with more robust systems may gain a marginal, albeit indirect, advantage in timely response.
- * **Market positioning implications:** Companies promoting products or services aligned with value-based care or specific modalities (like home dialysis) may need to adjust messaging following ETC termination. The focus for bundled products remains heavily influenced by the PPS structure and TDAPA pathway.

STRATEGIC RECOMMENDATIONS

Based on the analysis, the following strategic recommendations are proposed:

Immediate Actions (0-6 months):

- * **Form a dedicated, cross-functional CMS-1830-P Task Force:** Include representation from Strategy, Regulatory Affairs, Market Access, Commercial, Medical Affairs, Legal & Compliance, and R&D.; Charter the task force to analyze the final rule, drive implementation planning, and ensure timely communication across the organization.
- * **Conduct detailed Impact Modeling for Pipeline Assets:** Specifically assess the potential impact of the 3-year TDAPA window (effective 2028) on key pipeline molecules targeting the ESRD space. Identify critical decision points for development and launch timing.
- * **Develop and Submit Comprehensive Comments to CMS:** Prepare detailed comments on the proposed rule, focusing on the TDAPA eligibility modification (advocating for flexibility or grandfathering if possible, or seeking clarification), aspects of the PPS update impacting provider economics, and providing constructive input on RFI topics (Health IT, future QIP). Target submission well before the deadline.
- * **Enhance Regulatory Intelligence Monitoring Capabilities:** Immediately assess current capabilities for accessing FederalRegister.gov and eCFR.gov. Develop a plan to integrate with official APIs, utilize third-party services, or implement robust manual processes to ensure timely and reliable access to final rules and other critical regulatory information. Allocate necessary resources.

* **Engage Key Stakeholders:** Initiate outreach to major dialysis provider networks and patient advocacy groups to understand their perspectives on the proposed rule and implications for patient care and provider operations.

Medium-term Strategy (6-18 months):

* **Integrate Final Rule into Market Access Strategy:** Upon publication of the final rule, update market access and reimbursement strategies for relevant pipeline and marketed products. This includes specific plans for generating evidence and submitting TDAPA applications for eligible future products within the required timeframe (effective 2028).

* **Develop and Deliver Internal Training:** Create comprehensive training programs for Commercial, Medical Affairs, and other relevant teams on the implications of the final rule, including changes to payment, QIP, TDAPA, and ETC termination. Ensure compliant messaging.

* **Refine Regulatory Monitoring Implementation:** Fully implement chosen solutions (APIs, third-party services, manual processes) for reliable regulatory intelligence. Establish SOPs and training for relevant teams.

* **Monitor Competitive Actions:** Track how competitors adjust their strategies, particularly regarding pipeline announcements, launch timing, and messaging in the ESRD space.

Long-term Vision (18+ months):

* **Optimize Portfolio Strategy:** Continuously evaluate the R&D; pipeline and business development opportunities in nephrology based on the evolving reimbursement and policy landscape, including the long-term implications of the TDAPA change and the absence of the ETC model.

* **Proactive Engagement on Future Policy:** Actively engage with CMS, CMMI, and other policymakers on potential future policy developments, particularly those signaled by the RFIs (Health IT, new quality metrics), positioning Amgen as a constructive partner in improving ESRD care.

* **Strengthen Competitive Positioning:** Leverage deep understanding of the ESRD market dynamics, informed by policy analysis, to maintain and enhance Amgen's leadership position in nephrology.

RISK ASSESSMENT & MITIGATION

Regulatory Risks:

* **Non-compliance with TDAPA timing for future products:** If a pipeline product is FDA approved on or after Jan 1, 2028, and a TDAPA application isn't submitted within 3 years, eligibility is lost. This is a ****Critical**** risk for affected future assets.

* ***Mitigation:** Incorporate the 3-year clock into R&D; planning and market access strategy for relevant molecules early in development. Establish clear internal process owners and timelines for TDAPA data generation and submission post-approval.

* **Submission delays or rejections (TDAPA):** Risks inherent in any submission.

* ***Mitigation:** Ensure high-quality data packages and robust justification for future TDAPA applications. Engage in early dialogue with CMS if possible regarding future products.

* **Delayed awareness of final rule or other critical regulatory changes:** Operational risk due to Federal Register access challenges, potentially leading to missed comment deadlines or delayed

implementation of compliance obligations. ****Major**** operational risk.

* ***Mitigation:*** Invest in robust, reliable regulatory intelligence solutions (APIs, third-party services) or significantly enhance manual monitoring processes with dedicated resources. Establish clear internal communication channels and escalation procedures for regulatory intelligence alerts.

* **Inspection findings related to regulatory awareness/compliance:** Potential, though less direct from this rule's content.

* ***Mitigation:*** Maintain thorough documentation of regulatory monitoring processes and compliance activities.

Business Risks:

* **Suboptimal market access/reimbursement for future kidney assets:** If TDAPA timing is missed or strategy isn't effectively adapted. ****Major**** business risk for future revenue potential.

* ***Mitigation:*** Aggressive market access planning, potentially generating supportive evidence earlier in development. Close coordination between R&D; and Commercial/Market Access.

* **Negative impact on dialysis provider finances affecting sales volume:** While proposed increase is positive, QIP penalties persist. Economic pressure on providers could impact purchasing decisions. ****Major**** business risk.

* ***Mitigation:*** Monitor provider financial health. Provide strong value messaging for Amgen therapies. Explore potential partnership opportunities that align with provider needs and CMS incentives (e.g., related to RFI topics like health IT).

* **Competitive disadvantage:** If competitors more effectively adapt to the TDAPA change or navigate the post-ETC landscape.

* ***Mitigation:*** Rigorous competitive intelligence. Agility in strategy adaptation. Strong execution on market access and commercial plans.

* **Resource constraints:** Implementing new strategies, enhancing regulatory systems, and conducting detailed analysis requires resources.

* ***Mitigation:*** Prioritize initiatives based on risk and strategic importance. Secure necessary budget and personnel allocation for the task force and system enhancements.

Mitigation Strategies (Consolidated):

* **Proactive Stakeholder Engagement:** Formal comments to CMS, ongoing dialogue with dialysis providers, patient groups.

* **Robust Internal Processes:** Establish cross-functional task force, clear SOPs for strategy development, regulatory intelligence, and training.

* **Investment in Technology/Services:** Secure reliable access to regulatory information.

* **Integrated Planning:** Ensure alignment between R&D;, Regulatory, Market Access, and Commercial for pipeline strategy and launch timing.

* **Continuous Monitoring:** Track policy developments, competitive actions, and market dynamics closely.

* **Contingency Planning:** Develop alternatives if primary strategies face roadblocks (e.g., if TDAPA submission is delayed).

CONCLUSION & NEXT STEPS

The proposed CY 2026 ESRD PPS rule presents a complex mix of policy updates, most notably the critical change to the TDAPA eligibility timeline for future drugs and the termination of the ETC Model. These changes, while primarily impacting dialysis providers, have significant implications for Amgen's strategy in the nephrology space, particularly for our pipeline and future market access. The operational challenge of accessing regulatory information adds a layer of complexity and risk.

Our strategic priorities must focus immediately on understanding the full implications of the proposed rule, particularly the TDAPA modification, and preparing robust comments for submission to CMS. Simultaneously, we must address the regulatory intelligence access issue to ensure timely awareness of the final rule and other critical developments. In the medium to long term, we will integrate these changes into our pipeline planning, market access strategies, and commercial execution.

Key Success Metrics and KPIs:

- * Submission of high-quality, timely comments to CMS.
- * Successful implementation of enhanced regulatory intelligence capabilities.
- * Integration of the new TDAPA timeline into pipeline decision-making processes (e.g., reflected in development stage gates, target TPPs related to market access data).
- * Development and readiness of updated market access strategies for relevant pipeline assets.
- * Timely and effective training of Commercial and Medical teams on policy changes.
- * Maintaining or growing market share/sales volume for key nephrology products amidst evolving provider economics and modality trends.

Executive Team Action Items:

- * Approve the formation and resourcing of the CMS-1830-P Cross-Functional Task Force.
- * Review and approve Amgen's official comments on the proposed rule.
- * Approve investment in enhanced regulatory intelligence capabilities.
- * Monitor progress on integrating policy changes into pipeline and market access strategies.

Board Reporting Recommendations:

- * Provide regular updates on the evolving US healthcare policy landscape in key therapeutic areas, including nephrology.
- * Report on the strategic implications of the final ESRD rule, particularly concerning pipeline assets and market access strategies.
- * Detail the steps taken to address operational risks related to regulatory monitoring.
- * Present updated risk assessments and mitigation strategies related to these policy changes.

By proactively analyzing these proposed changes, engaging with stakeholders, and adapting our internal strategies and operations, Amgen can effectively navigate the evolving ESRD landscape and continue to deliver value to patients and providers.
